AM-03 Dialysis / Renal Failure

MEDICAL-DIRECTOR APPROVED APP CONTENT | SOURCE-PDF REPLACEMENT | Reconciled October 16, 2026

Scope: Use for renal-failure or dialysis complications including hyperkalemia, pulmonary edema, sepsis, shock, access hemorrhage, or peritoneal-dialysis concern.

Hyperkalemia, access, and fluid-risk pathway

1. Assess + monitor	ABCs, ECG/12-lead, glucose, last dialysis/missed sessions, access findings, respiratory status, and perfusion.
2. Hyperkalemia concern?	Weakness, paralysis, syncope, bradycardia, missed dialysis, peaked T waves, QRS widening, or unstable dysrhythmia.
3. ECG instability	Calcium gluconate 1.5-3 g IV (preferred peripheral) OR calcium chloride 500-1,000 mg IV/IO. Do not give both as the initial dose.
4. Shift potassium	Albuterol 10 mg nebulized for hyperkalemia with a pulse; do not use for hyperkalemic cardiac arrest. Bicarbonate 1 mEq/kg, max 50 mEq, only for significant ECG instability, metabolic acidosis, or hyperkalemic arrest.
5. Fluid risk	No routine fluid for renal failure/missed dialysis/hyperkalemia. For hypotension without pulmonary edema, NS 250 mL IV/IO with reassessment; maximum 500 mL without Medical Control.
6. Access hemorrhage	Direct, focused pressure. Tourniquet proximal to the access only for life-threatening bleeding uncontrolled by direct pressure.
7. Peritoneal dialysis	Keep transfer set/catheter capped and clamped as found. Do not manipulate, flush, disconnect, drain, or collect effluent.
8. Transport	Bring dialysis information when available and notify early for hyperkalemia, pulmonary edema, sepsis, shock, or access hemorrhage.

Key treatment and disposition

Item	Approved action
Line compatibility	Do not mix calcium and sodium bicarbonate. Flush the line thoroughly between administrations.
Persistent shock	Use AM-05 for norepinephrine rather than repeated large-volume crystalloid.

Calcium stabilizes the myocardium but does not remove potassium. Definitive potassium removal generally requires dialysis.

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SAFETY, LINKED PATHWAYS, AND RELEASE CONTROL

Safety checks

- Calcium extravasation can cause severe tissue injury; monitor the site continuously. Calcium gluconate is preferred through a peripheral IV.

- Do not place routine equipment or vascular access in the fistula/graft extremity when another site exists.

- A dialysis-access hemorrhage can cause rapid exsanguination. Effective bleeding control takes priority over access patency.

- Claiborne EMS does not administer insulin for hyperkalemia under AM-03.

Linked pathways

Pathway	Use when
AC-01	Nonshockable arrest with suspected hyperkalemia
AC-09	VF/pulseless VT with suspected hyperkalemia
AC-02	Symptomatic bradycardia
AC-05	Pulmonary edema
AM-05	Persistent shock

Documentation and release control

- Document focused findings, BP/rhythm/ECG when applicable, all interventions with dose/route/time/response, serial reassessments, destination decision, and notifications.

- This source PDF replaces the legacy imported sheet and matches the approved Claiborne EMS app content reconciled October 16, 2026.

- Use the linked protocol whenever the patient crosses into another clinical pathway. Follow agency release governance, training requirements, and current medication/device availability.